

The status of children health services (global and national situations)

CHILD HEALTH PROBLEMS & SERVICES

09/23/26

Objective

- At the end of this session, students will be able:
- To define childhood
- To describe child health problems
- To identify child survival interventions
- To describe child health services

Brain storming

- What are the major child health problems?

Child Definitions

"every human being below the age of 18 years "

Biologically, a child is anyone in the developmental stage of childhood, between infancy and adulthood.
[UNCRC (Child act of 2001)]

Stages after birth

- The newborn also known as the **neonate** from birth to the first 28 days of life.
- „ The **infant**: birth up to one year of life;
- „ The **young child** (Preschool child): from the age of one year to five years of life.
 - The child is also referred to Under 5 child.
- „ The school child is the child 5 to 15 years
- „ The final stage is the Adolescent who is a child aged 10 to 19 years of age (WHO, 1987).

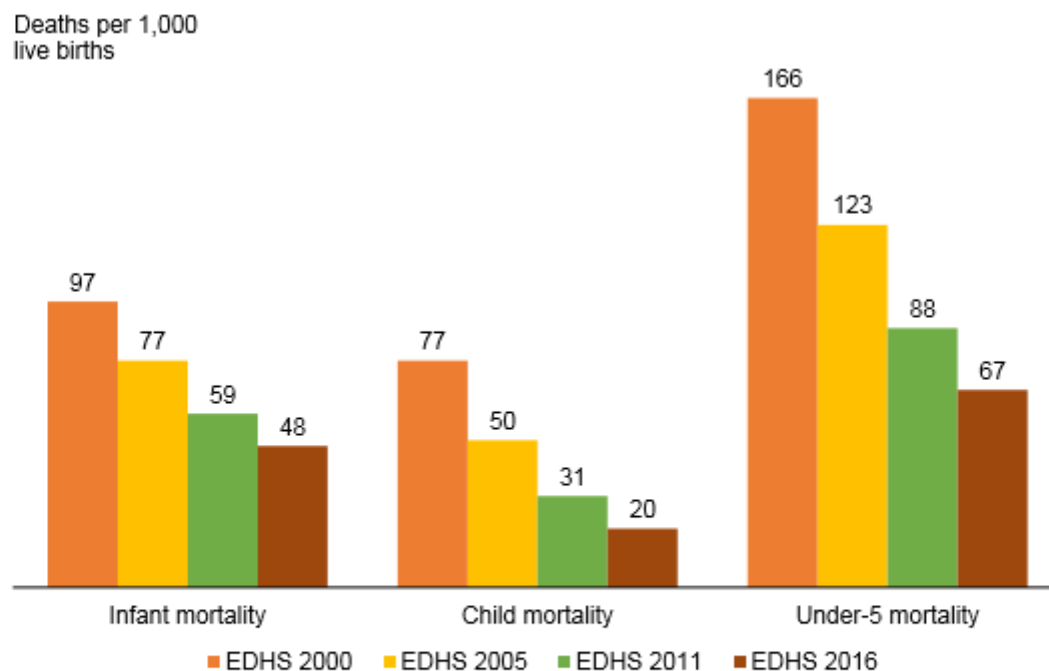
child health problems

Global situation

- Over **22,000** children die **every day** around the world(2009).
- That is equivalent to:
 - **1** child dying every **4** seconds
 - **15** children dying every minute
 - Just **under 8.1** million children dying every year
 - Some **88** million children dying between **2000 and 2009**

Figure 4 presents trends in childhood mortality in Ethiopia since the 2000 EDHS survey. Data show that there has been a steady decline in infant, child, and under-5 mortality over the last 16 years. For example, under-5 mortality rates for the 5 years preceding the survey declined from 166 deaths per 1,000 live births to 123 deaths per 1,000 live births in 2005, to reach 67 deaths per 1,000 live births in 2016. Similarly, infant mortality decreased from 97 deaths per 1,000 live births, to 77 deaths per 1,000 live births, and to 48 deaths per 1,000 live births in the same period.

Figure 4 Trends in childhood mortality, 2000-2016



- for approximately 1 million children in 2015, their first day of life was also their last.
- Globally, the neonatal mortality rate is declining less rapidly than the mortality rate for children between 1 month and 5 years of age.
- This means that the share of under-five deaths occurring during the neonatal period is increasing.
- in 2015, neonatal deaths accounted for 45 per cent of total deaths, 5 per cent more than in 2000

- in sub-Saharan Africa, newborn deaths account for about one third of the deaths of children under age 5.
- in 2015, about 80 per cent of these deaths occurred in South Asia and sub-Saharan Africa, and almost half occurred in just five countries: the democratic republic of the Congo, Ethiopia, India, Nigeria and Pakistan.

Child health problems...

Africa situation

- In sub-Saharan Africa it has been estimated that every minute **eight** U5 children die
- Of the 46 countries in the African Region,
 - **36** have U5 mortality rates of above 100 per 1000 live births;
 - **8** have U5MRs of at least 200 per 1000 live births;
 - **5** countries have had static U5MRs in the past fifteen years while
 - **9** countries the U5MRs have reversed.

Child health problems...

- **Two thirds** of the U5 deaths in the African Region are due to easily preventable & treatable diseases.
- The chief causes of death are
 - **neonatal conditions** and
 - acute respiratory infections mainly **pneumonia**,
 - **malaria**,
 - **diarrheal diseases**,
 - **measles** and **HIV/AIDS** ,
 - most of which are complicated by **malnutrition** that accounts for **one third of all** deaths in children under five years.

Child health problems...

- The highest rates of mortality in children U5 years old continue to occur in sub-Saharan Africa, which accounted for **half of** child deaths worldwide in 2008
 - **1 in 7** children in the region died before their fifth birthday.
 - South Asia accounted for **one third** of child deaths in 2008.
- According to WHO, an estimated 10.6 million under-five children die each year, 4.6 million of whom die in the African Region.

Child health problems...

- Food insecurity in Africa threatens the lives of millions of vulnerable people especially displaced persons and PLWHA
- Under nutrition is directly or indirectly responsible for
 - 3.5 million child death every year, and
 - at least 35% of the disease burden in U5 children.
- Sub-Saharan Africa has one of the highest prevalence of low birth weight ranging from 7-42%. Exclusive breastfeeding rate is low and complementary foods are inadequate and inappropriate in the region.

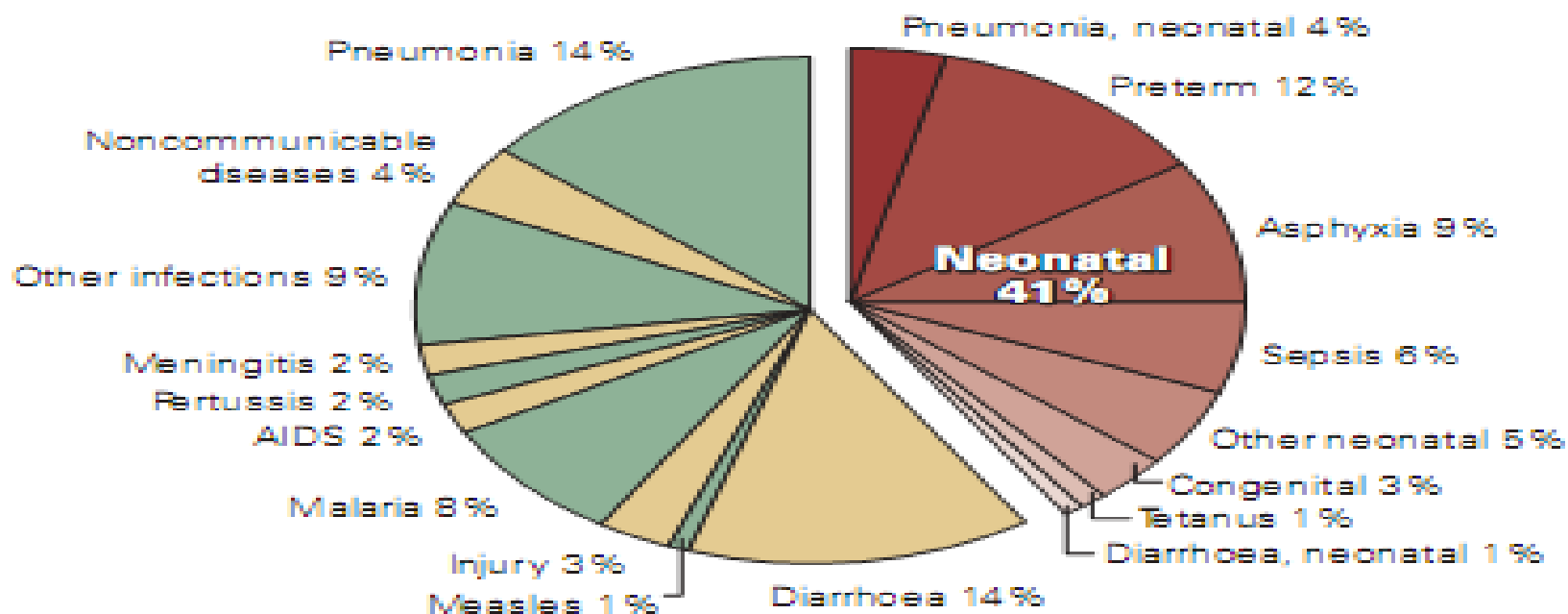
Child health problems...

- According to WHO 2007, Annually
 - almost one million children die from pneumonia,
 - 769, 000 from diarrhea and
 - a further 810, 000 from malaria;
 - 600, 000 children under five years are infected with HIV each year, mainly through MTCT of HIV, and
 - 315, 000 die from AIDS.

Child death ...

More than 40% of child deaths occur during the neonatal period

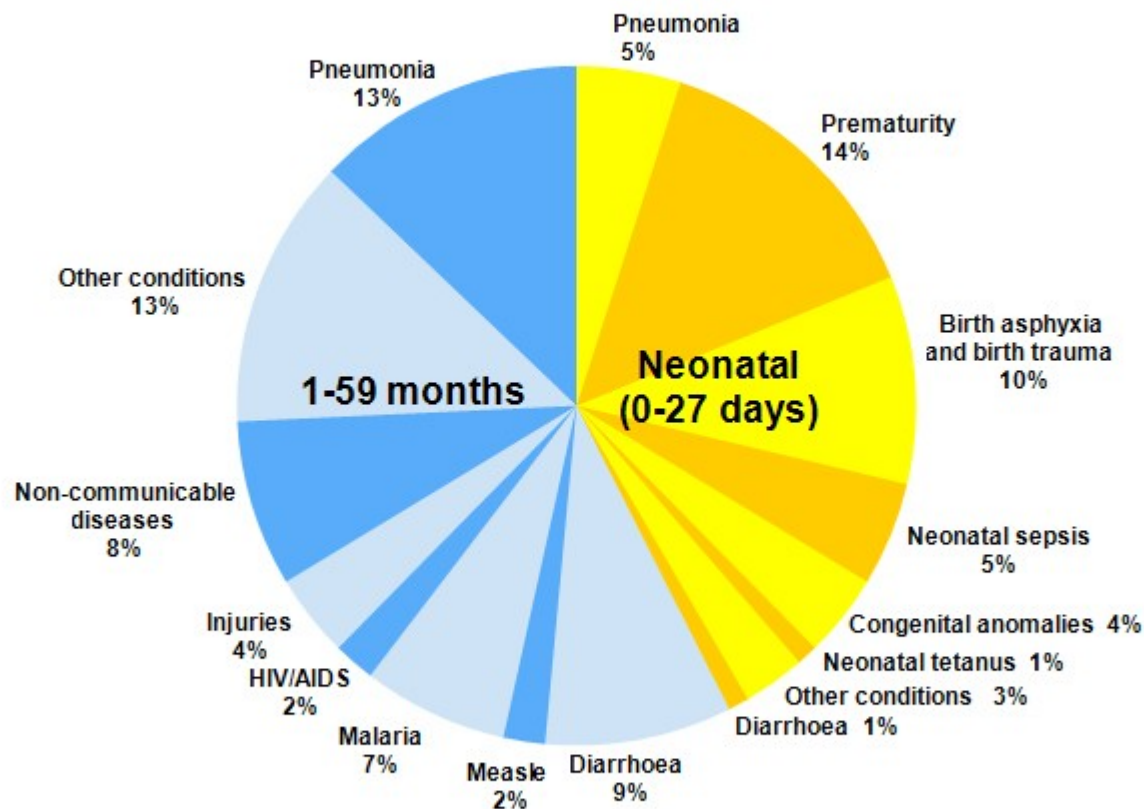
Global causes of death among children ages 0–59 months, 2008



Undernutrition contributes to one-third of child deaths.

Source: Black and others forthcoming.

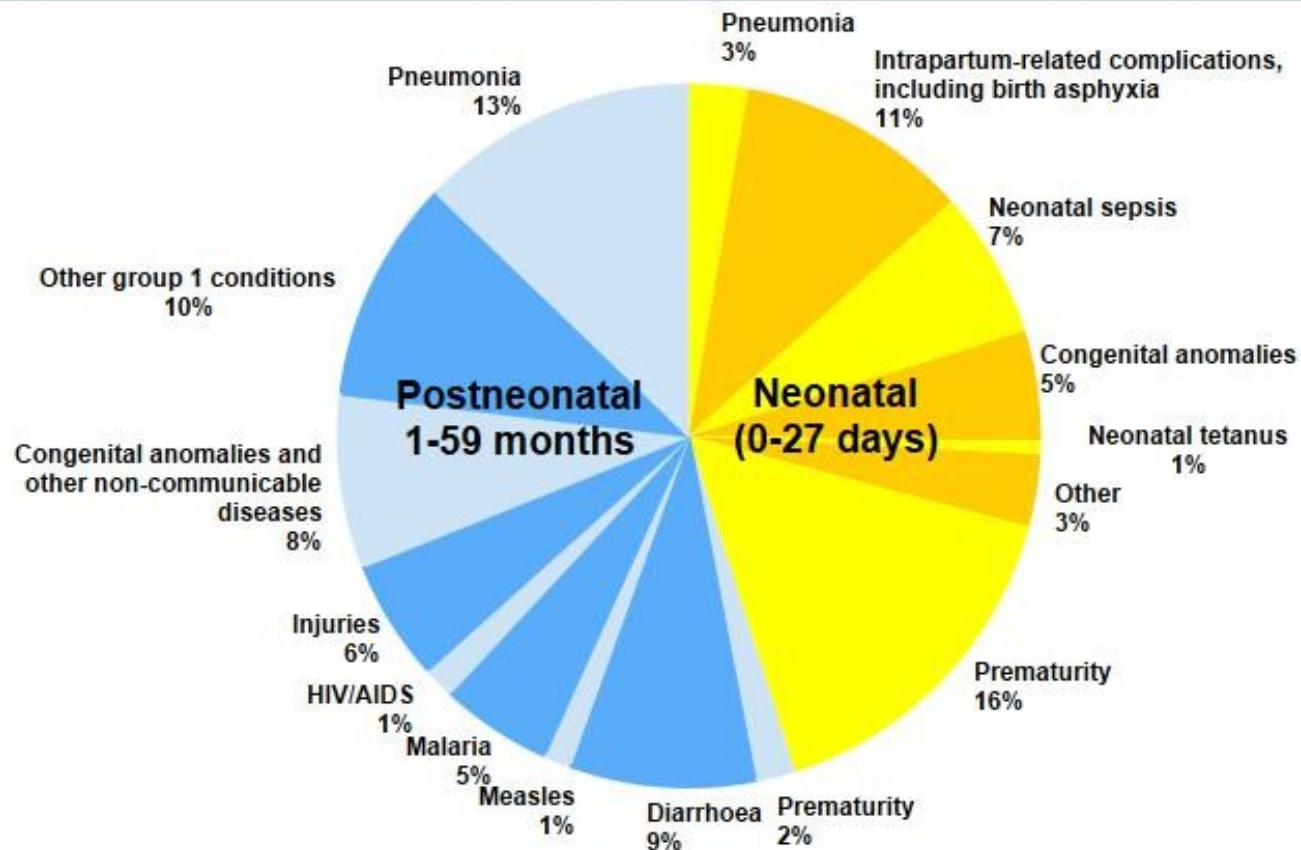
Causes of deaths among children under 5 years, 2011



Source: WHO-UNICEF Child Health Epidemiology Reference Group (CHERG) estimates, 2013.

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Causes of deaths among children under 5 years, 2015

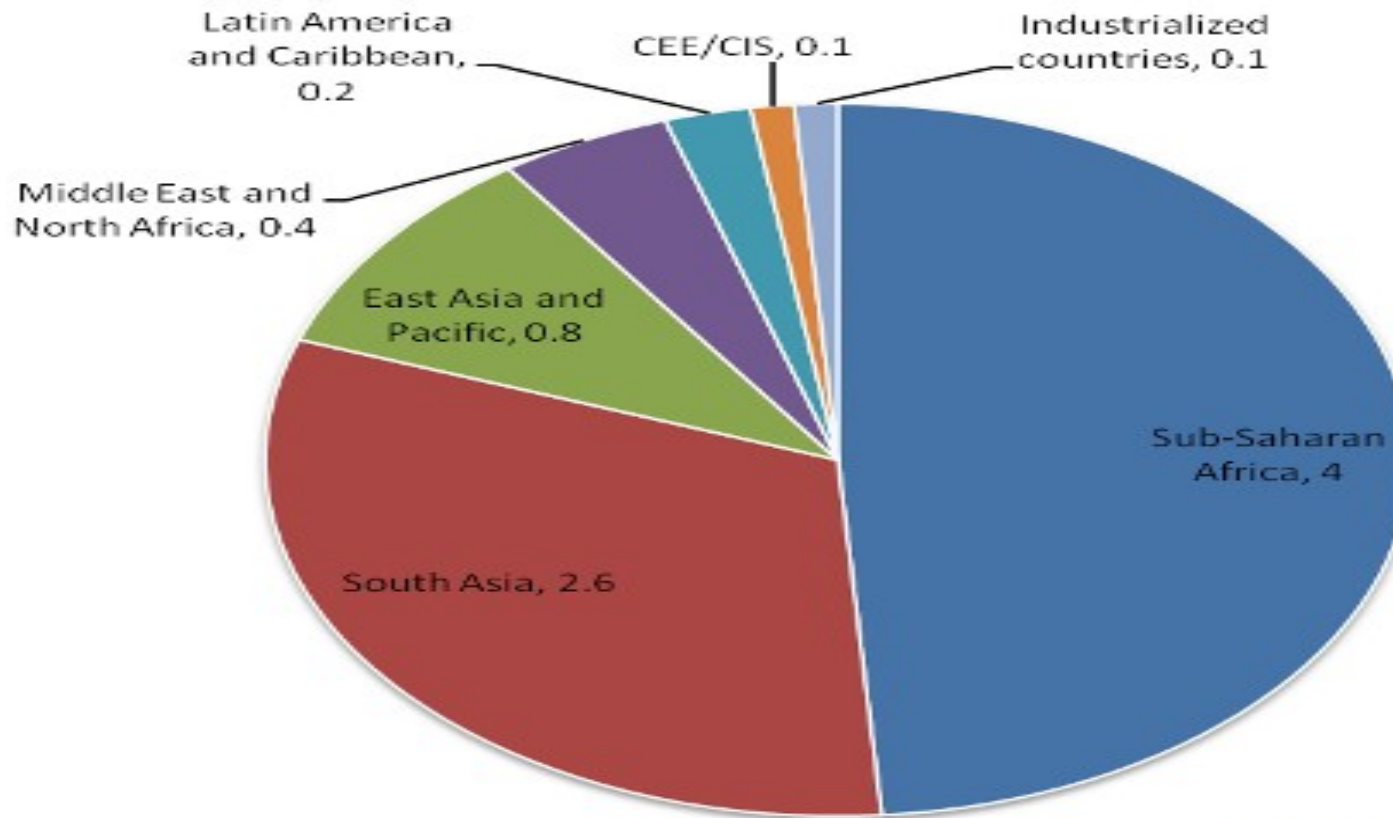


Source: WHO-MCEE methods and data sources for child causes of death 2000-2015
(Global Health Estimates Technical Paper WHO/HIS/IER/GHE/2016.1)

Child death...

www.globalissues.org

Under 5 Child Deaths in 2009 (millions)

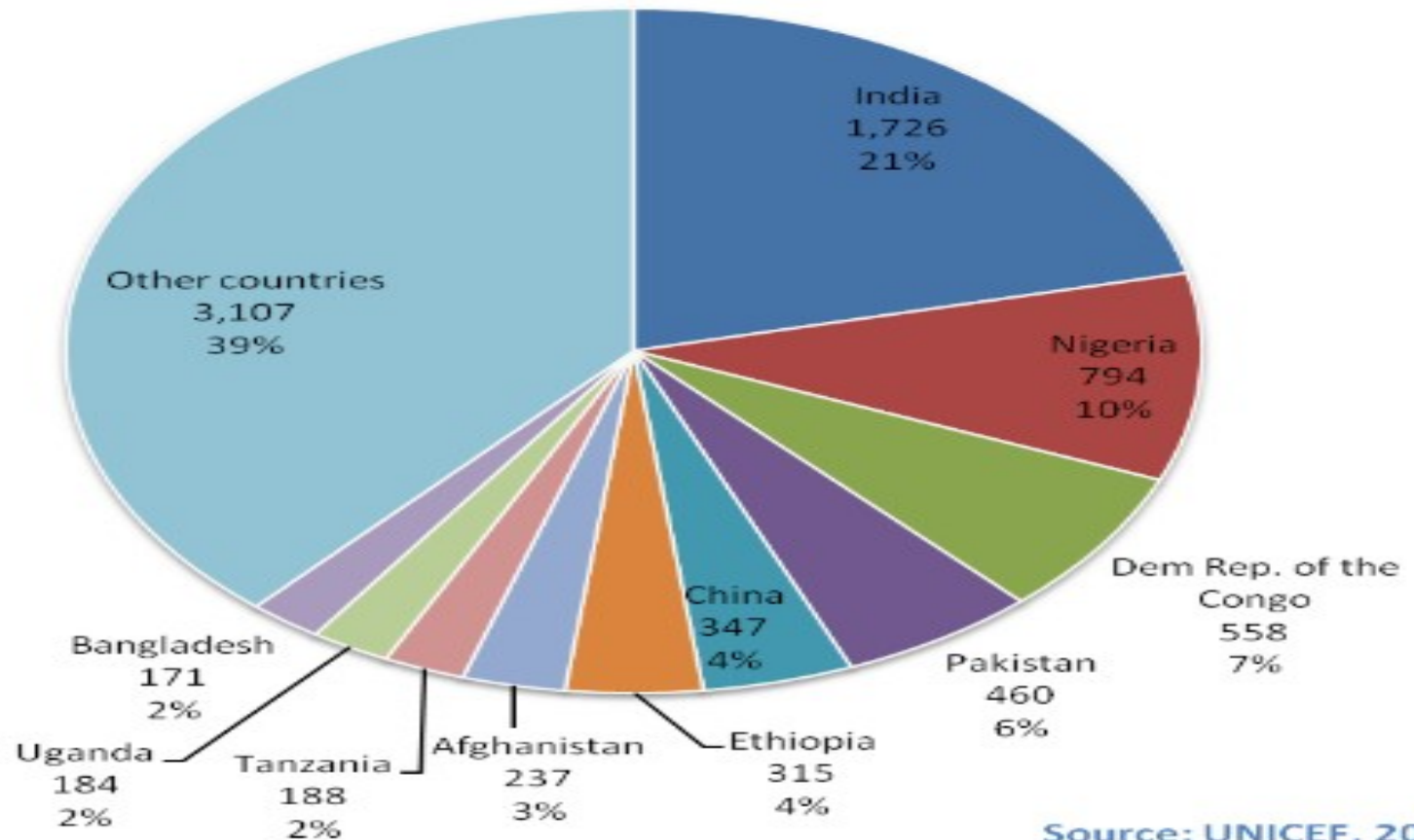


Source: UNICEF, 2010

Child death...

www.globalissues.org

Top 10 countries with the most under-5 deaths in 2009 (thousands)



Source: UNICEF, 2010

Why is child mortality important to understand?

- The U5 mortality rate has several advantages as a barometer of child well-being in general and child health in particular.
- First, **it measures an ‘outcome’ of the development process rather than an ‘input’**, such as per capita calorie availability or the number of doctors per 1,000 population—all of which are means to an end.
- Second, **the U5MR is known to be the result of a wide variety of inputs:**
 - the nutritional status and the health knowledge of mothers;

Why is child mortality important

- the level of immunization and oral rehydration therapy;
- the availability of maternal and child health services
(including prenatal care);
- income and food availability in the family;
- the availability of safe drinking water and basic sanitation;
and
- the overall safety of the child's environment, etc

Why is child mortality important ...

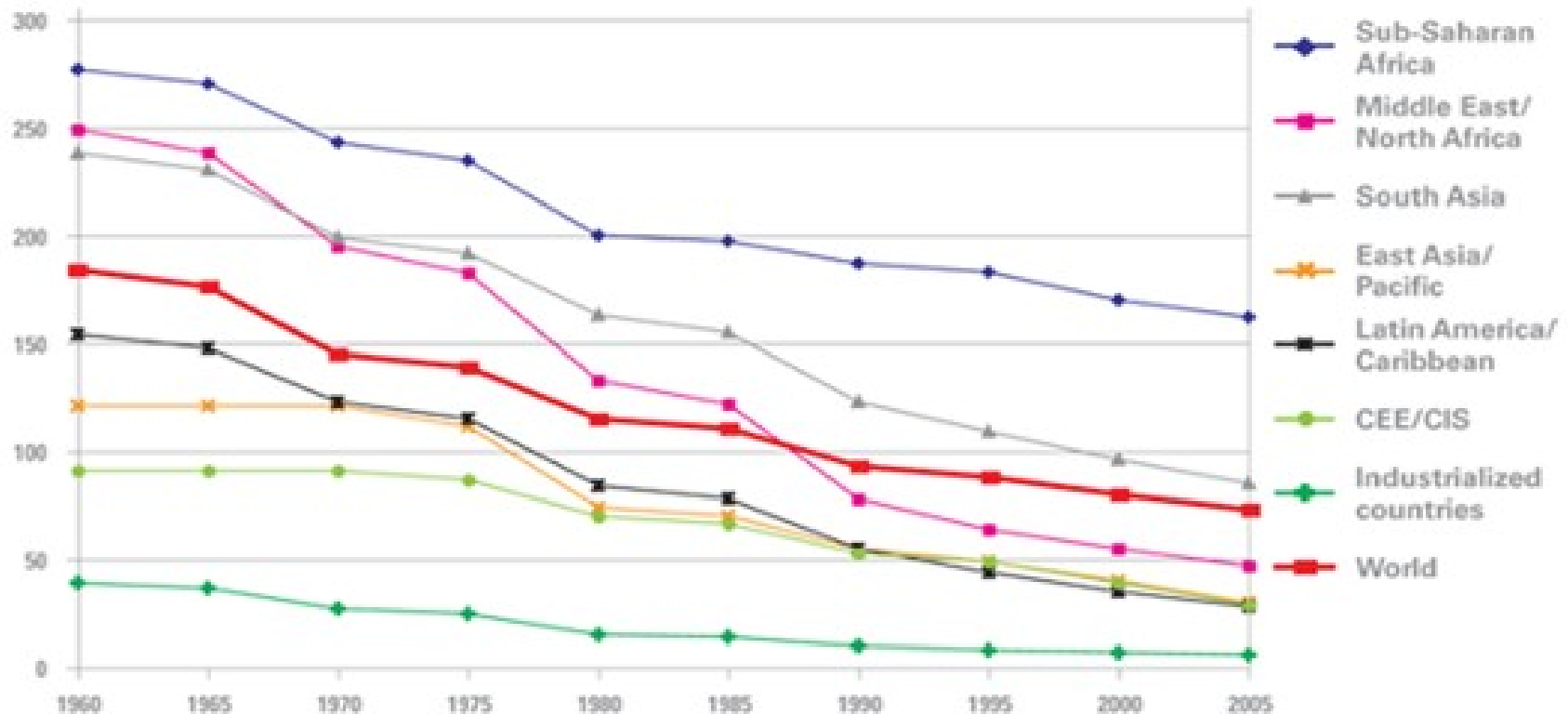
- Third, **the U5MR is less susceptible to the fallacy of the average** than, for example, per capita gross national income (GNI per capita).
 - In other words, it is much more difficult for a wealthy minority to affect a nation's U5MR, and it therefore presents a more accurate, if far from perfect, picture of the health status of the majority of children (and of society as a whole).

Trends in child mortality

- The global child mortality rate declined by over a third between 1990 and 2009:
- Progress has certainly been made as each year the number of children under 5 dying is slowly coming down
- progress has been unevenly distributed. For example, good progress was made by a few nations with large populations, but many countries made “no progress or insufficient progress”

Trends in child mortality...

Under-five mortality rate (per 1,000 live births), by region (1960–2005)



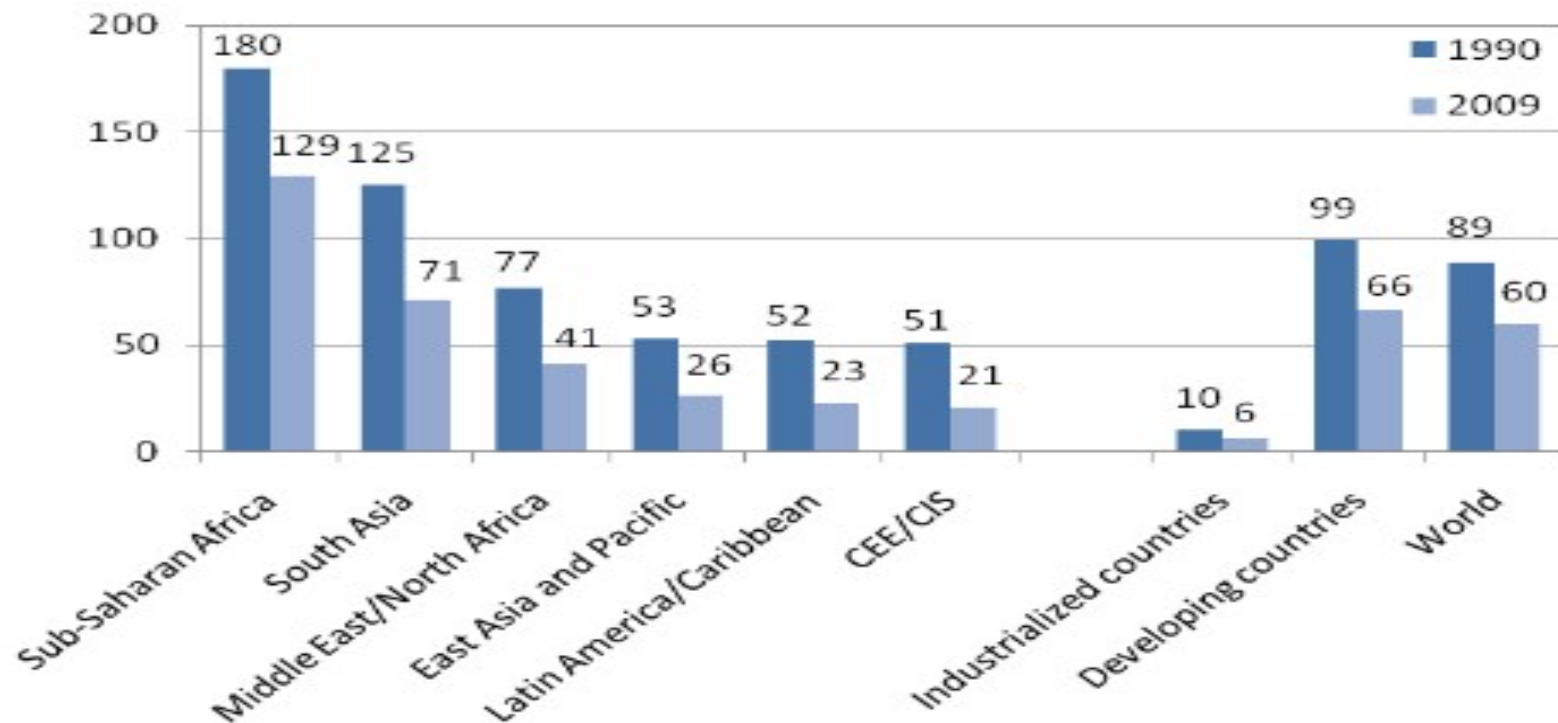
Source: UNICEF, Progress for Children 2007

Trends in child mortality...

www.globalissues.org

Decline in under-5 mortality rates, 1990-2009

Per 1,000



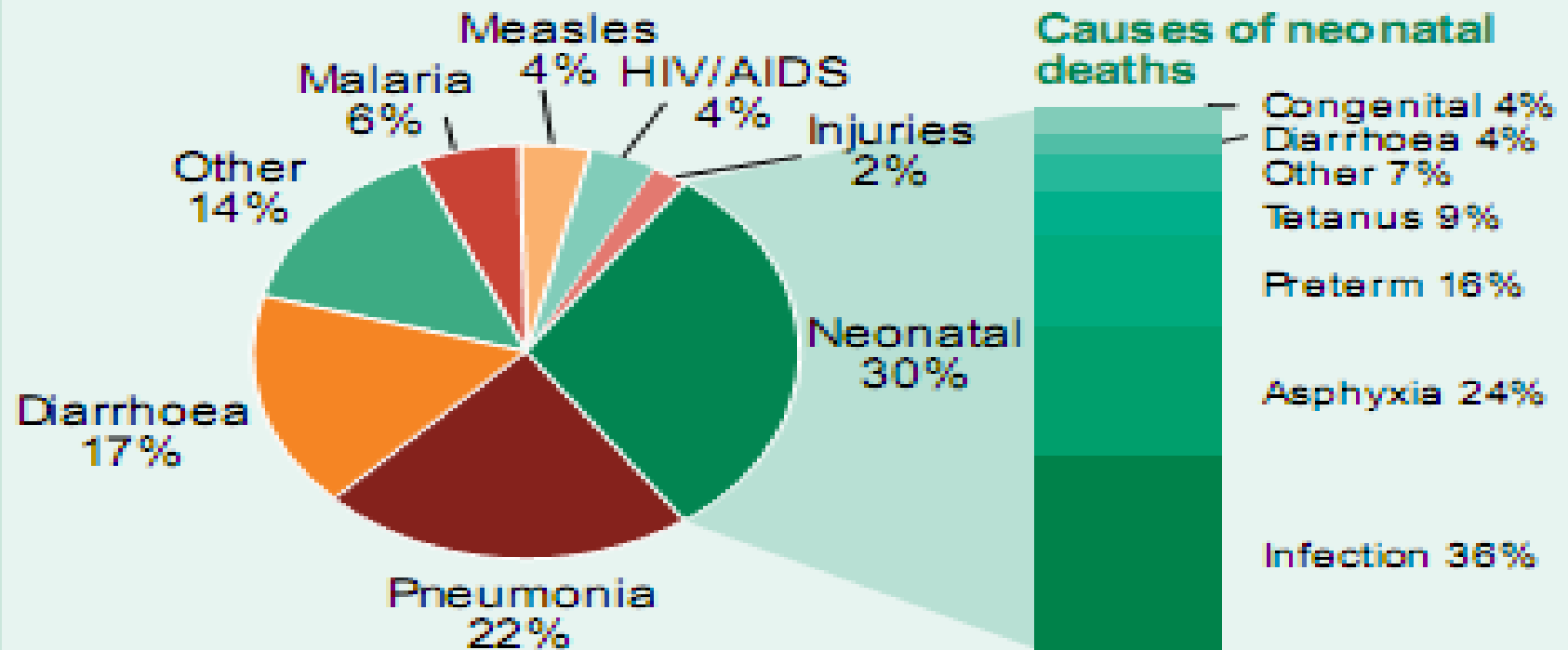
Source: UNICEF, 2010

Ethiopian situation

Child health problem in Ethiopia...

Causes of under-five deaths

Globally more than one third of child deaths are attributable to undernutrition

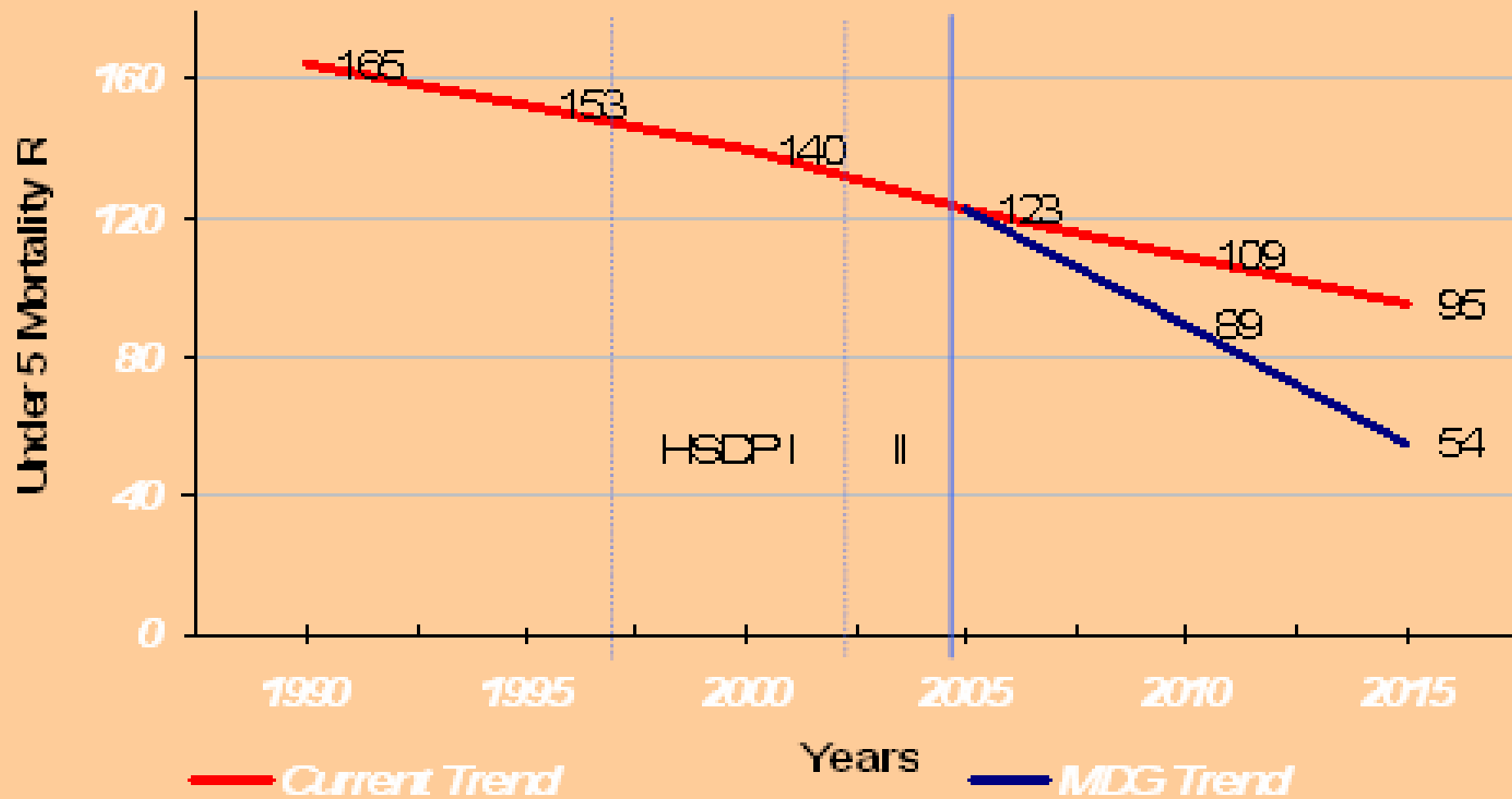


Source: WHO, 2008

Source: Lawn JE, Cousens SN for CHERG (Nov 2008)

Acceleration to reach MDG 4 (1,5,6)

No business as usual



Trends of Under Five Mortality Rate Since 1990

1990 1995 2000 2005 2010 2015

- Current Trend 165 153 140 123 109 95
- MDG Trend 165 153 140 123 89 54

Reductions in mortality rate

- Ave. annual reduction (1990-2000) -2.5
- Ave. annual reduction (2000 - 2005) -3.4
- Ave. annual reduction (1990 - 2005) -2.8
- Ave. annual reduction (2005 - 2015) to
achieve the MDG -6.9

Child Survival

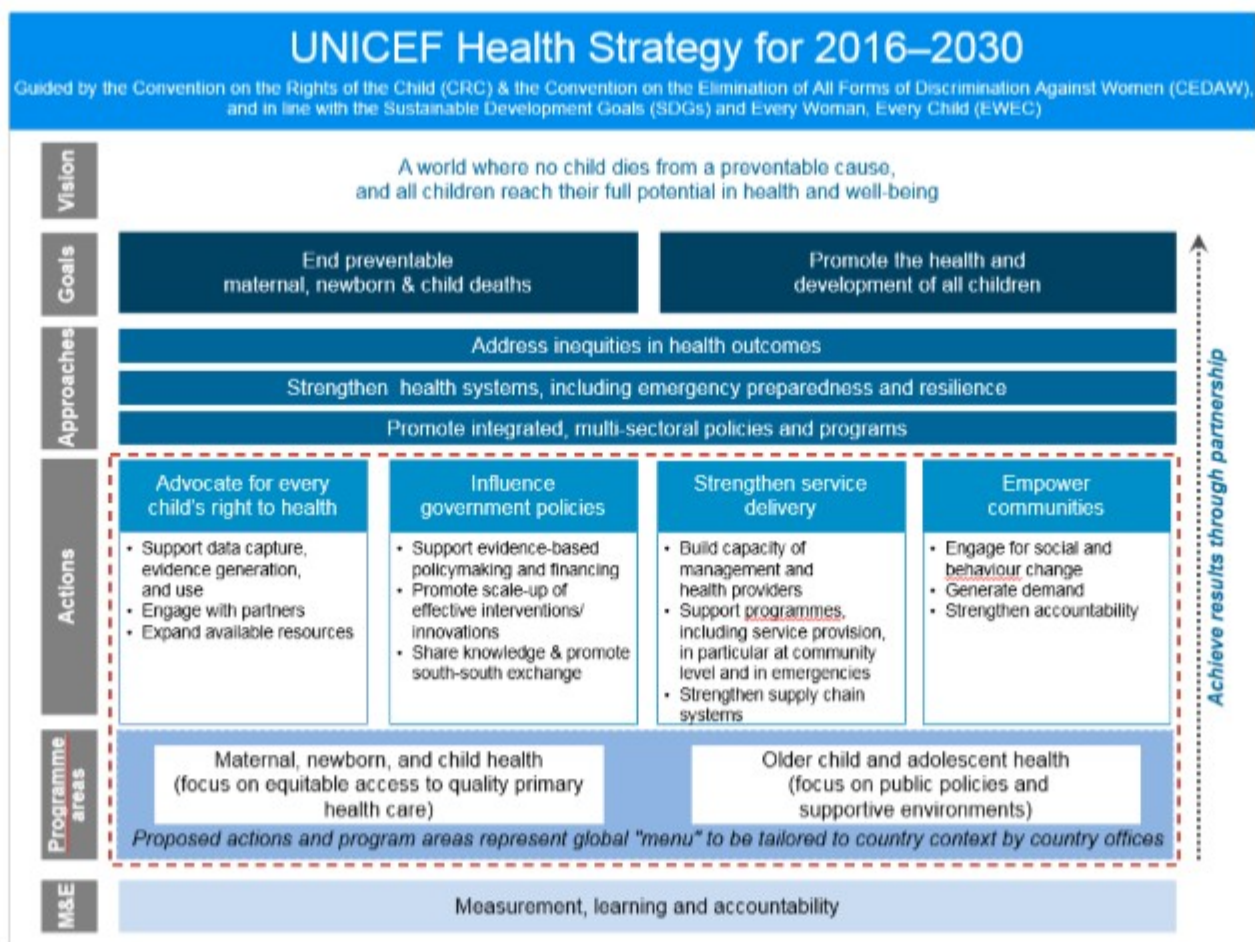
❖ For the initial five year period 2016-2020, UNICEF's Strategy for Health (hereafter called "the Strategy") sets two overarching goals:

- 1. End preventable maternal, newborn and child deaths**
- 2. Promote the health and development of all children.**

To achieve these goals, the Strategy considers the health needs of the child at all life stages.

In order to increase focus and coherence across health programmes, the Strategy identifies three approaches (addressing inequities in health outcomes, strengthening health systems including emergency preparedness, response and resilience⁶, and promoting integrated, multisectoral policies and programmes)

Figure 1: Overview of UNICEF Strategy for Health (2016-2030)²⁰



Priority child survival interventions

- Evidence shows that there are a number of known and affordable interventions that if implemented fully could prevent 63% of current childhood mortality.
- The priority child survival interventions that need to be implemented at scale fall within the area of:
 - ❑ [Newborn care](#), taking into consideration the life cycle approach and continuum of care;
 - ❑ [Infant and young child feeding](#), including micronutrient supplementation and deworming;

child survival interventions...

- ❑ Provision of maternal and childhood [immunization](#) and promotion of new vaccines;
- ❑ Prevention of mother-to-child transmission ([PMTCT](#)) of HIV;
- ❑ Prevention of [malaria](#) using insecticide-treated nets (ITNs) and intermittent preventive treatment of malaria (IPT);
- ❑ Management of common childhood illnesses and severe acute malnutrition, including at community management where applicable using the [Integrated Management of Childhood Illness](#) (IMCI) strategy;
- ❑ Management and care of HIV-exposed or infected children

child survival interventions...

- At the programmatic and policy levels,
 - It is not enough simply to expand the delivery of packages of low-cost, proven interventions: **behavioral**, **institutional** and **environmental obstacles** that can impede access must also be addressed as part of the scaling-up process.
- Success requires an in-depth understanding of these obstacles, as well as of the strategies **for circumventing** them.

child survival interventions...

- **Approaches** for ensuring child survival outlined in the strategy are:
 - advocating for harmonization of child survival goals and agenda;
 - strengthening health systems;
 - empowering families and communities;
 - building operational partnerships, and
 - the mobilization of resources at all levels to scale up proven interventions.

Delivery approaches /System

- Aim is to achieve coverage of quality care at various health service contact points
 - Health Extension Program
 - ... Integrated Management of Childhood Illness
 - ... Immunization plus
 - ... Nutrition Program
 - ... Making Pregnancy Safer
 - ... Malaria Control Program/RBM/
 - ... HIV/AIDS
 - ... Environmental Health (water, hygiene, sanitation)

Main Challenges

- Inadequate access to health facilities
- Shortage of skilled human resources at all levels, including community
- Lack of essential equipment and supplies
- Inadequate motivation of personnel
- Inadequate supervision, HMIS

Child health services

- AIMS
 - Promote healthy development
 - Prevent illness
 - Treat minor ailments
 - Immunization
 - Nutritional surveillance & advice.
 - Refer major ailments

Child health services...

- Activities are
 - Screening,
 - Growth monitoring,
 - Immunization (EPI),
 - Control of Diarrheal Diseases (CDD) and
 - Acute Respiratory Infections (ARI) need to be included or considered.

Child health services...

- Child health service components are:-

- ☐ Well-baby clinic

- ☐ Screening

- ☐ Growth monitoring

- ☐ Immunization

- ☐ Sick baby clinic

- ☐ Day care centre services

- ☐ Care for the handicaps

- ☐ School health care

- ☐ Adolescent health care

Screening

- is making a rapid assessment of the needs of a child
- is a very important activity which has to be carried out every day in all institutions delivering child health services
- Avoids missed opportunities for:
 - Vaccination
 - Growth monitoring
 - Other cares related with acute problems
- The purpose of screening is to identify children or mothers for appropriate treatment or care

The Screening Process

- Observe general health status
 - Weight for age (GM chart)
 - Vital signs
- Observe immunization status
- Observe for acute problems such as
 - Neglect and physical abuse
- Observe for acute problems which need urgent treatment or referral to higher institution

Growth and Development

- **Growth and development** are continuous dynamic processes occurring from conception to maturity, taking place in an orderly sequence that is similar in all persons.
- **Growth**
 - Is the gradual increase of the body and consists of increase in cell numbers and their size
 - Depends upon adequate nutrition and normal health
 - A child who grows well is probably healthy and adequately nourished

Growth and Dev't...

Development

- The increase of physical and mental skills and capacities includes:
 - Maturation of organs and systems
 - Acquisition of physical, intellectual and interpersonal skills
 - Ability to adapt more rapidly to stress
 - Capacities for creative expression

Growth Monitoring and Promotion

- **Growth Monitoring**

- Regular monthly recording weight of the children and plotting it on the growth chart which enables us to see the changes in the weight and giving advise to mother about the growth of child.

- **Growth promotion**

- Is giving the mother of such a child relevant and practical guidance within her means or that of the family or the community on what to do to ensure growth continues well or resumes, in case it was not proceeding normally.

Growth Monitoring

- The process of GM combines:
 - Identification,
 - communication or education for the individual mother, the family, and the community on matters related to the growth, treatment of illness and improvement of nutrition.

Importance of GM & P

○GM is a strategy which enables the health workers:

- To assess normal growth of infants and young children
- To detect deviation from normal growth and determine the hidden problem
- To make right decision regarding the interventions required
- To provide appropriate information to the mother or family about the child's condition

○GM also helps the mother to have regular contact with primary health services.

Day Care Center Services

- Child care is a term used to describe short-term care that is taken up by someone other than a parent.
- Day care center – an institution caring for normal children (pre-school) who have no access of care at home

Day Care Center

Supervision of day care centers should focus on:

- Screening for health problems
- Growth monitoring
- Nutrition
- Hygiene inspection
 - Personal hygiene
 - Environmental sanitation
- Conducive playground and adequate rooms

Care for Disabled Children

- Disability is an umbrella term covering impairments, activity limitations, and participation restrictions.
- Traditionally their problem is believed to be due to some divine punishment for some sin made by the parents.

Cause for Disabled Children...

○ Physical handicap is mostly related with

- Perinatal asphyxia
- Birth trauma
- Burns
- Car accident
- Infection – e.g. polio

○ Visual impairment

- Vitamin A deficiency

○ Hearing impairment

○ Repeated infection of middle ear - measles

Care for Disabled Children...

- Early detection of disability with early intervention to prevent secondary problems will reduce the risk of further disability and help the child to develop his/her full potential.
- Disabled children need special care beside the routine health care. They need:
 - Emotional and psychological support
 - Conducive environment
 - Skill training for daily life

Care for Disabled Children

Measures to be taken

○ Educate the public on major causes

○ Educate the mother about

- Infection prevention
- Nutrition
- Importance of immunization
- Early diagnosis and treatment of childhood illness
- Provision of basic health services to mothers & children at all level
- Collaborate work with other ministries (MOE, etc)

School Health Care

○ Promotive services

- Health education
- Physical exercise

○ Preventive services

- Immunization
- Growth monitoring
- Screening....

○ Curative services

- First aid
- Treatment of minor illness
- Follow up

School Health Care

- A health promoting school is one that constantly strengthens its capacity as a healthy setting for living, learning and working.
- In addition to clinical services, the school physical features should be appropriate for the health of children

Thank you!!!!!!!!!!